جوستاف روسي انترناشيونال - مصر (EPE) *

طلب علاج أول مرة / متردد

إسم المريض رباعياً : فاطمه ابوالحديد فوزى ابوالحديد
المحافظة : القليوبيه **الحي / المدينة :** القناطر الخيريه **العنوان :** شبرا الخيمه اول
رقم البطاقة : 28205191400662 **رقم التليفون :** 01208581107 **تاريخ تسجيل الإستمارة :** 23-10-2026
العمر(تاريخ الميلاد) : 19-05-1982 **المهنة :** ربه منزل

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:-التأمين الصحي-القوات المسلحه-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي **المقر بما فيه**
التوقيع:(----------) **التوقيع:**(----------)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)
نتائج الأبحاث : (مرفق)
التشخيص النهائي : سرطان بالثدي (مبادرة)
خطة العلاج : أبحاث أورام مزيد من الابحاث و الاشعات [7954] **قسم العلاج :** الاورام
التعليق على التشخيص : ----------

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/ندى حسن **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/نرمين رمضان **الوظيفة:** ---------- **التوقيع:** ----------

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية.
كما تقر اللجنة بأن البيانات المدونة فى هذا النموذج صحيحة.

تعتمد صحة الإجابات المرفقة
رئيس اللجنة و مدير المستشفي.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفي قبل الصرف.

أعضاء اللجنة

ا.د/ ---------- **ا.د/** ---------- **د/** ---------- **د/** ----------

قرار اللجنة

التشخيص النهائي : ----------
خطة العلاج : ----------
المستشفي : ----------**قسم :** ----------
جهة العلاج : ----------**نفقات :** ----------
ملاحظات : ----------

رئيس	مدير عام	ختم
المجلس الطبي المتخصص	مديرية الشئون الصحية	شعار الجمهورية

Name: فاطمه ابوالحديد فوزى ابوالحديد
National ID: 28205191400662

تقرير طبي

السيد

تحية طيبة وبعد,,

إيماء للطلب المقدم من	بشان تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بان نوضح فيما يلي تقريرا عن حالته

تاريخ وساعة الدخول : ---:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026

وبعد الكشف والفحوصات والمتابعة وجد انة

المريضة تعاني من ورم غير حميد بالثدى و تحتاج الي Further investigations and imaging

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين





٠١٢٠٨٥٨١١٠٧

— ماعنده أم
— موظف/موظفة
— كبادة
— مدرس/مدرسة

جوستافروسي

مبادرة رئيس الجمهورية لدعم صحة المرأة المصرية

البنك المصري لتنمية الصحة

MDT FORM 16/11/2025 G

Name	فاطمة بوالخير	MRN	٢٧٥٨١٢٥
		ID	01208581105
Age	43 years.	Phone	01152221597
Menopausal status	Peri Menopausal	Marital status	Married.
NRS SCORE		Family history	+ve = Breast
BMI		REASON OF MDT	علاج السرطان
CO morbidity	-ve		٤٨٢٠٥١٩١٤٠٣٢

Clinical Data

BX: IDC + 5% DCIS.

ER:	PR:	HER 2:	KI67:
8/8	5/8	+3	25%

Sonomammography: ACR B

Rt UOQ at 10 o'clock 8 cm o'clock

Mets Workup: 42 x 22.5 mm & 13 x 12 mm + Multiple level 1 Pathological Lymph Node

TNM T2 N1

CTCAP: Free . bone scan free

Recommendations: for Neoadjuvant + Ojaimekian then will be + Sentinel LND according to post NACT Histopathology

مراجعة ورعاية ٢٠٢٥/١٠/٢٠

MDT OUTCOME & PLAN

- for Clipper of lesions 9-10 o'clock
- NACT ___ TCBP 6cycle
- then Rt WLE + SLNB acc to Response

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric

Name: فاطمه ابوالحديد فوزي هليل	
MR. : 2712	

Detailed Medical Report

السيد	

تحية طيبة وبعد,,

إيماء للطلب المقدم من		بشأن تقرير طبي للمريض	
بقسم:		رقم طبي:	

نشرف بأن نوضح فيما يلي تقريرا عن حالته

تاريخ وساعة الدخول :		تاريخ وساعة الخروج :	

وبعد الكشف والفحوصات والمتابعة وجد انه

The patient was diagnosed with primary breast cancer, clinical stage T2 N1, invasive ductal carcinoma grade 2, estrogen receptor 8/8 positive, progesterone receptor 5/8 positive, HER2 3+ positive. Surgical margins and sentinel nodes were negative. Radiotherapy was planned. The patient received two cycles of Taxotere (docetaxel) combined with Carboplatin AUC 5, followed by PHESGO administered in cycle 3. The patient has completed 2 cycles of Taxotere/Carboplatin and is currently on cycle 3 of PHESGO.

On subsequent evaluation, no palpable breast masses were noted. The patient exhibited grade 2 onychomycosis. The treatment plan was to proceed with cycle 4 of Taxotere 135 mg and Carboplatin AUC 5, and to continue PHESGO in cycle 3. The patient has completed cycle 4 of Taxotere/Carboplatin and cycle 3 of PHESGO.

The patient's most recent chemotherapy cycle was not received; however, PHESGO was administered in cycle 3 for the upcoming cycle 4 Taxotere/Carboplatin regimen, and a second dose of PHESGO was given for Solucortef with an increased infusion time. The patient has received cycles 3 and 4 of PHESGO.

For cycle 5, the patient was scheduled to receive Taxotere/Carboplatin (AUC 5) and PHESGO, with laboratory work pending and an echo scheduled. Methylprednisolone was added for grade 2–3 peripheral neuropathy, and the infusion rate was extended to over 2 hours. The patient has completed cycle 5 of Taxotere/Carboplatin and PHESGO.

On examination, a right upper outer quadrant mass measuring 2 × 2 cm was noted without palpable axillary lymph nodes. The patient continued to have grade 2 onychomycosis and required supportive treatment. The plan was for cycle 6 of Taxotere 120 mg with Carboplatin AUC 5 and PHESGO, along with prophylactic Neupogen; however, chemotherapy was delayed pending multidisciplinary team (MDT) approval for surgery. The patient has completed cycle 6 of Taxotere/Carboplatin and PHESGO.

The patient was prepared for surgical consultation after receiving cycle 6 of Taxotere 120 mg with Carboplatin AUC 5 and was scheduled to receive PHESGO in cycle 7, together with one dose of prophylactic Neupogen. The patient has completed cycle 6 of Taxotere/Carboplatin and cycle 7 of PHESGO.

تحريرا في ,

التاريخ :	18-09-2026	الوقت :	
الطبيب المعالج :	dina makram		

يعتمد ,

STAVE/
IUSSY
ATIONAL

Patient Name	: الأء.ذ / فاطمه ابو الحديد فوزي ابو الحديد	Request No.	: 13009754
Patient No.	: 282051914003662	Request Date	: 17/09/2025
Path No.	: 2844/25	Verification Date	: 22/09/2025
Sex	: Female	Financial Category	: جوستلف روسي - مباشر
Age	: 43 Y	Department	: Out Patient
Referring Doc	:	Registered by	: Mrs.Mayada Hegazy

Pathology Report

Clinical Data: Right breast 10 o'clock malignant solid mass.
Nature of Specimen: Tru-cut biopsy, collected on 16/09/2025.

Gross Examination:
Three grey white tissue cores, each was 1.5 cm long. Totally submitted.
Technique:
Sections prepared from paraffin block for Immune-staining using Bench Mark ULTRA system for:
ER clone (1E2), PR clone (SP1), Her2 clone (4B5) & Ki67 clone (30-9).

Microscopic Examination:
Histological type: Invasive duct carcinoma.
Grade 2 (differentiation 3, nuclear pleomorphism 2, mitosis 2).
5% Intermediate grade ductal carcinoma insitu (Solid & cribriform patterns).
Angioinvasion: Absent.
Perineural Invasion: Absent.
Peritumoral lymphocytic infiltrate: 10%.

IPT (Positive control showed appropriate reactivity):
ER: Strong nuclear reaction in 90% of tumor cells (score 8/8).
PR: Strong nuclear reaction in 5% of tumor cells (score 5/8).
Her2: Strong complete membrane reaction in more than 10% of tumor cells (score 3+).
KI67%: 25%.

Diagnosis:
Right breast mass, Tru-cut biopsy:
Invasive duct carcinoma, Grade 2.
5% Intermediate grade ductal carcinoma insitu (Solid & cribriform patterns).

Verified by :

غرفة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

STAVE / TUSSY

Patient Name	: [illegible] / فاطمه ابو الحديد فرزي ابو الحديد	Request No.	: 13009754
Patient No.	: 28205191400662	Request Date	: 17/09/2025
Path No.	: 2844/25	Verification Date	: 22/09/2025
Sex	: Female	Financial Category	: جروبتك رومي - مباشره
Age	: 43 Y	Department	: Out Patient
Referring Doc :		Registered by	: Mrs.Mayada Hegazy

Pathology Report

PT:
- ER: Positive (score 6/8).
- PR: Positive (score 5/8).
- Her2: Positive (Score 3+).
- KI67%: 25%.

' Scoring system: "ER, PR"
ASCO-CAP ER/PR Test Guideline Recommendations: proportion of cells staining (0, no staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66%
and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, weak, 2, intermediate; and 3, strong). Total score from 0 to 8.
' Scoring system: "Her2"
Reference: ASCO-CAP Her2 Test Guideline Recommendations, 2013.IHC 0: as defined by no staining observed or membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within > 10% of the invasive tumor cells. IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within >10% of
the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is complete, intense > 10% of tumor cells.

Signature
Dr. Walaa Ibrahem Ghonem.
Dr. Duaa Mohamed.

Verified by :

غرفة رقم (4) - الدور الأول - مبنى رقم (937) - فورلوش النيل - مصر القديمة - القاهرة - مصر
Room No. (4) - First Floor-Building No. 937- Nile Corniche - Misr El Qadema - Cairo, Egypt

Pathology Report
(DOCTYPE PATHOLOGY_REPORT)

GUSTAVE ROUSSY
INTERNATIONAL

Patient Name: Fatma Abu Al Hadeed Fawzy Abu Al Hadeed	Date : 01/09/2025
Exam : ultrasound BREAST (BOTH SIDE)	Age : 43 Year(s)
Medical ID: 8157	Pat. ID: 111232

BILATERAL BREAST MAMMOGRAPHY AND COMPLEMENTARY ULTRASOUND

INDICATION:
Right breast palpable lump

TECHNIQUE:
- Bilateral MLO and CC views
- Bilateral breast ultrasound

REPORT:
- Fairly dense fibro glandular density, (ACR B).

RIGHT BREAST:
- The UOQ shows two ill-defined masses, associated with asymmetry, consistent with the clinical palpable lump.
- Complementary U/S revealed underlying solid hypoechoic masses located at 10 and 9 o'clock, measuring 42x22.5 and 13x12mm respectively, associated with surrounding altered parenchymal texture. Vascularity was detected within both masses on color Doppler application.
- Multiple level I pathological axillary lymph nodes are also seen reaching 26x15mm.
- Free infra and supra clavicular regions.

LEFT BREAST:
- No dominant mass, spiculation or suspicious micro calcification.
- Complimentary U/S examination depicted no solid or cystic lesion.
- Nonspecific axillary node.

OPINION:
- Right UOQ multifocal neoplastic process, (BIRADS 5), for biopsy.
- Normal left breast, (BIRADS 1).

Best regards,
Dr. Mahi Mokhtar

AVICENNA
POUSSY
INTERNATIONAL

Patient Name: Fatma Abu Al Hadeed Fawzy Abu Al Hadeed	Date : 16/10/2025
Exam : CT CAP+C (CT Chest Abd. Pelvis)	Age : 43 Year(s)
Medical ID: 2758/2025	Pat. ID: 111232

MULTISCLICE C.T. EXAMINATION OF THE CHEST, ABDOMEN AND PELVIS AFTER I.V. CONTRAST INJECTION REVEALED:

Reported case of right breast cancer showing:

I- **CHEST:**

- Right breast ill-defined soft tissue mass lesion seen measuring about 2.7x2.9 cm associated with pathologically enlarged ipsilateral axillary lymph node largest measuring about 2.1cm in its short axis.
- No sizable pulmonary nodules, masses, consolidation, collapse or cavitary lesions.
- No sizable hilar or mediastinal masses or lymph node enlargement.
- No pleural or pericardial collection.
- No gross cardiomegaly.

II- **ABDOMEN AND PELVIS:**

- Average sized liver showing homogenous parenchymal attenuation. No focal lesion or dilated biliary radicles.
- Normal C.T appearance of the spleen, both kidneys, adrenal glands, pancreas, aorta and IVC.
- The urinary bladder shows no evidence of gross masses or diverticulae or stones.
- Normal appearance of the pelvic organs with no pelvic masses or collections.
- No ascites.

Opinion:

- Reported case of right breast cancer showing right breast ill-defined soft tissue mass lesion associated with pathologically enlarged ipsilateral axillary
- No CT evidence of sizable metastatic pulmonary nodules
- No CT evidence of sizable abdomen or pelvic metastases.

Dr. Nourhan Mohamed, MD

SCAN & LABS

مركز النيل للأشعة و التحاليل

Name : Fatima Abul-Hadid Fawzi
Branch : Dokki
Ref.Physician Prof : Gostaf Roseh Hospital

Date : 01-11-2025 10:34
Medical No. : 238659
Ins.Company : بدون شركة تأمين

$Tc^{99}m$ MDPSKELETAL SCINTIGRAPHY

History

- A 70-year-old patient, with history of breast cancer and referred for metastatic workup.

Technique

- Anterior and posterior whole-body scans were acquired 2 hours post injection of the tracer.

Findings

- No abnormal active or cold focal bony lesions that could account for metastatic bone disease.
- Degenerative changes are noted at the spine and big joints.

Impression:

- **The skeleton is free from metastases.**

Best regards,
Dr. Alaa Nagi, MD
Nuclear Medicine



جمهورية مصر العربية
بطاقة تحقيق الشخصية

فاطمه
ابو الحديد فوزي ابو الحديد
١٢ ش الاعلمين - متفرع
شبرا الخيمة اولى - القليوبية

٢٨٢٠٥١٩١٤٠٠٦٦٢

JZ9016747



٢٨٢٠٥١٩١٤٠٠٦٦٢ ٠٥/٢٠٢٣
بدون عمل

انثى مسلمة متزوجة
عبدالرحمن محمد عبدالعزيز ابراهيم
١١ البطاقة سارية حتى ١٥/٠٥/٢٠٣٠



11ccc1097 :ت

DMS
تعيين
ايكو + تدعيم كامل
ماقبل اجراء

GUSTAVE ROUSSY INTERNATIONAL

جوستاف روسي انترناشونال مصر

ECHO REPORT

NAME : فاطمة ابو الحمد فوزي DATE: 29/7/2026

ID NUMBER : CVON /C-CO SEX : AGE:

EF	62%	55-75%	PWT	0.8	0.6-1.1cm
LVEDD	4.8	3.5-5.8cm	AO.R	2.3	3-3.7cm
LVESD	2.5	2-4cm	LA	3.6	2-4cm
FS	28%	25-45%	RV		0.7-2.3com(supine)
SWT	0.9	0.6-1.1cm	AO.CUSP		1.5-2.6cm

LV Dimensions Nml

LV Wall Thickness Nml

LV Diastolic Function Nml

SWMA No RWMA

LA and Ao Root Dimensions Nml

Mitral Valve trivial MR

Aortic Valve N

Tricuspid Valve trivial TR

Pulmonary Valve

Right Side and Systolic Function TAPSE: 2.3

Intracardiac Masses and Effusion

DR.

اسم المريض بالإنجليزية: Fatemah Abo Alhadid Fawzi	كود المريض: 97776
تاريخ التقرير: 2026/08/03 11:11:41 AM	السن: ٤٤ سنة, ٢ شهر

Stereotactic Guided Wire localization:

Clinical summary: Pathologically proven right breast cancer underwent NACT for pre-operative stereotactic wire localization over 2 UOQ clips.

Technique:

- Under complete sterile conditions, 3ml of local anaesthetic were administered at the site of concern in the right breast.
- Two 21G wires were used to localized 2 clips.
  - **Wire (1):** The wire is located at right 9:00 over **Clip (1).** The wire is inserted perpendicular from the upper aspect for about 27 mm and to reach the clip with the hook is seen 6 mm superficial to the clip.
  - **Wire (2):** The wire is located 10:00 over **Clip (2).** The wire is inserted perpendicular from the upper aspect for about 35 mm and below the skin for 11.6 mm to reach the clip with the hook is seen 4 mm deep to the clip.
- This was Confirmed by CC and MLO views.

Much Obliged
Dr. Amal Hegazi, M.Sc.

GUSTAVE ROUSSY
INTERNATIONAL

وستاف روسي

ECHO REPORT

NAME : فاطمة ابوالحديد DATE: 7 /12/2025

ID NUMBER : / SEX : AGE: 42

EF	70 %	55-75%	PWT	0.8	0.6-1.1cm
LVEDD	4.8	3.5-5.8cm	AO.R	2.3	3-3.7cm
LVESD	2.3	2-4cm	LA	3.5	2-4cm
FS	31 %	25-45%	RV		0.7-2.3com(supine)
IVT	0.9	0.6-1.1cm	AO.CUSP		1.5-2.6cm

LV Dimensions Normal
LV Wall Thickness Normal
LV Diastolic Function Normal
SWMA No RWMA
LA and Ao Root Dimensions Normal
Mitral Valve N
Aortic Valve
Tricuspid Valve N
Pulmonary Valve
Right Side and Systolic Function TAPSE: 2.5
Intracardiac Masses and Effusion

DR.

Patient ID: 90333	Patient Name English: Fatemah Abo aldid Fawzi Abo Alhadid
Age: ٣٢ سنة, ٢ شهر	2025/11/27 10:40:18 AM :Report Date

Right Ultrasound guided clip insertion

Indication:

Right breast:

- Under complete aseptic condition & ultrasound guidance two clips were inserted for right breast two malignant mass lesions
- Frist lesion between 8-9 o'clock measures 10x8mm
- Second lesion between 9-10 o'clock measures 27x20mm
- the clip was inserted at the center of both lesion and this was confirmed by mammographic CC & ML

Best Regards,

Dr. Marwa M.Hassan



جوستاف روسي

Patient Name	: فاطمه ابوالحديد فوزى	**Patient Id**	: 3624
Patient Info.	: Female 43Y	**VisitId**	: 5702
Department	: Out Patient	**Collection Date**	: 07/12/2025 11:25:56
Profile NO.	: 2758-2025	**Reporting Date**	: 07/12/2025 11:31:06

COMPLETE BLOOD PICTURE

Test		Result	Unit	Reference Range
CBC				
Hemoglobin (HB)	:	13.1	g/dL	12 - 16
RBCs	:	**H** 4.87	10^6/cmm	3.8 - 4.8
Haematocrit (HCT)	:	41.4	%	36 - 46
MCV	:	85.0	fl	76 - 95
MCH	:	26.9	pg	26 - 32
MCHC	:	**L** 31.6	g/dL	32 - 36
RDW	:	12.10	%	11.6 - 14.5
Platelet count	:	368	10^3/cmm	150 - 450
WBCs	:	8.40	10^3/cmm	4 - 10

Differential white cell Count	*Relative*		*Absolute*	
	Relative (%)	*R.Range*	*Absolute (x 10^3/ul)*	*R.Range*
Neutrophils	65.50	40 - 70	5.50	2 - 7
Lymphocytes	26.90	20 - 45	2.26	1 - 3
Monocytes	3.30	2 - 8	0.28	Up To 1.o
Eosinophils	4.20	1 - 6	0.35	Up To 500
Basophils	0.10	0 - 1	0.01	Up To 100

Signature

Dr. Hassan Sayed Fadl

Patient Name	**: فاطمه ابو الحديد فوزى**	**Patient Id**	**: 3624**
Patient Info.	**: Female 43Y**	**VisitId**	**: 5702**
Department	**: Out Patient**	**Collection Date**	**: 07/12/2025 11:26:06**
Profile NO.	**: 2758-2025**	**Reporting Date**	**: 07/12/2025 11:51:22**

Clinical Chemistry Report

Test		Result	Unit	Reference Range
Kidney Function				
Creatinine	:	0.50	mg/dL	0.46 - 0.83
Liver Function				
Total Bilirubin	:	0.45	mg/dL	0.1 - 1.2
SGPT (ALT)	:	12	U/L	7 - 40

Signature

Dr. Hassan Sayed Fadl

جوستافروسي انترناشونال مصر

مبادرة رئيس الجمهورية لدعم صحة المرأة المصرية

أنت المصرية هي صحة مصر

MDT FORM 16/11/2025 G

Name	فاطمة بوالحسين	MRN	CVONICO
		ID	01208581107
Age	43 years.	Phone	01152221597
Menopausal status	peri Menopausal.	Marital status	Married.
NRS SCORE		Family history	+ve. « Breast »
BMI		REASON OF MDT	New Case
CO morbidity	-ve		enc. 01914..77

Clinical Data

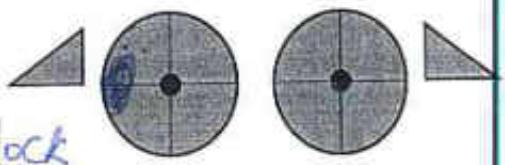
BX: IDC + 5% DCIS.

ER:	PR:	HER 2:	Ki67:
8/8	5/8	+3	25%

Sonomammography: ACR B
Rt UOQ at 10' oClock & 9 oclock

Mets Workup: 4.2 x 22.5 mm & 13 x 12 mm
+ Multible level 1 Pathological Lnod

CTCAP: free . bone scan free



TNM
2 1

Recommendations: for Neadjvent + elimination then wl E + SLNB or ALND according to post NACT Mammography

MDT OUTCOME & PLAN:
- for Clipping of lesions 9 o'clock
- NACT → TCHP 6cycles
- then Rt wLE + SLNB acc to response

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric

JSTAVE
DUSSY
NATIONAL

Patient Name	: الأستاذه / فاطمه ابوالحديد فوزي ابوالحديد	Request No.	: 13009754
Patient No.	: 28205191400662	Request Date	: 17/09/2025
Path No.	: 2844/25	Verification Date	: 22/09/2025
Sex	: Female	Financial Category	: جوستاف روسي - مبادره
Age	: 43 Y	Department	: Out Patient
Referreing Doc :		Registered by	: Mrs.Mayada Hegazy

Pathology Report

Clinical Data: Right breast 10 o'clock malignant solid mass.
Nature of Specimen: Tru-cut biopsy, collected on 16/09/2025.

Gr s Examination:
- T ee grey white tissue cores, each was 1.5 cm long. Totally submitted.

Technique:
- Sections prepared from paraffin block for immune-staining using Bench Mark ULTRA system for:
- ER clone (1E2), PR clone (SP1), Her2 clone (4B5) & Ki67 clone (30-9).

Microscopic Examination:
* Histological type: Invasive duct carcinoma.
* Grade 2 (differentiation 3, nuclear pleomorphism 2, mitosis 2).
* 5% Intermediate grade ductal carcinoma insitu (Solid & cribriform patterns).
* Angioinvasion: Absent.
* Perineural invasion: Absent.
* Peritumoral lymphocytic infiltrate: 10%.

* IPT (Positive control showed appropriate reactivity):
- E : Strong nuclear reaction in 90% of tumor cells (score 8/8).
- P : Strong nuclear reaction in 5% of tumor cells (score 5/8).
- Her2: Strong complete membrane reaction in more than 10% of tumor cells (score 3+).
- Ki67%: 25%.

Diagnosis:
Right breast mass, Tru-cut biopsy:
- Invasive duct carcinoma, Grade 2.
- 5% Intermediate grade ductal carcinoma insitu (Solid & cribriform patterns).

Verified by :

CAP ACCREDITED
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USTAVE
OUSSY
NATIONAL

Patient Name	: الأستاذه / فاطمه ابوالحديد فوزي ابوالحديد	Request No.	: 13009754
Patient No.	: 28205191400662	Request Date	: 17/09/2025
Path No.	: 2844/25	Verification Date	: 22/09/2025
Sex	: Female	Financial Category	: جوستاف روسي - ميادره
Age	: 43 Y	Department	: Out Patient
Referreing Doc :		Registered by	: Mrs.Mayada Hegazy

Pathology Report

IPT:
- ER: Positive (score 8/8).
- PR: Positive (score 5/8).
- H 2: Positive (Score 3+).
- K 7%: 25%.

* Scoring system: "ER, PR"
ASCO-CAP ER/PR Test Guideline Recommendations: proportion of cells staining (0, no staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66%
and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, week, 2, intermediate; and 3, strong). Total score from 0 to 8.
* Scoring system: "Her2"
Reference: ASCO-CAP Her2 Test Guideline Recommendations, 2013.IHC 0: as defined by no staining observed or membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within > 10% of the invasive tumor cells.
IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within >10% of
th invasive tumor cells or complete and circumferential membrane staining that is intense an within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is complete, intense > 10% of tumor cells.

Signature
Dr. Walaa Ibrahem Ghoneim.
Dr. Duaa Mohamed.

Verified by :

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GUSTAVE ROUSSY
INTERNATIONAL

Patient Name: Fatma Abu Al Hadeed Fawzy Abu Al Hadeed	Date : 01/09/2025
Exam : ultrasound BREAST (BOTH SIDE)	Age : 43 Year(s)
Medical ID: 8157	Pat. ID: 111232

BILATERAL BREAST MAMMOGRAPHY AND COMPLEMENTARY ULTRASOUND

INDICATION:
Right breast palpable lump

TECHNIQUE:
- Bilateral MLO and CC views
- Bilateral breast ultrasound

REPORT:
- Fairly dense fibro glandular density, (ACR B).

RIGHT BREAST:
- The UOQ shows two ill-defined masses, associated with asymmetry, consistent with the clinical palpable lump.
- Complementary U/S revealed underlying solid hypoechoic masses located at 10 and 9 o'clock, measuring 42x22.5 and 13x12mm respectively, associated with surrounding altered parenchymal texture. Vascularity was detected within both masses on color Doppler application.
- Multiple level I pathological axillary lymph nodes are also seen reaching 26x15mm.
- Free infra and supra clavicular regions.

LEFT BREAST:
- No dominant mass, spiculation or suspicious micro calcification.
- Complimentary U/S examination depicted no solid or cystic lesion.
- Nonspecific axillary node.

OPINION:
- Right UOQ multifocal neoplastic process, (BIRADS 5), for biopsy.
- Normal left breast, (BIRADS 1).

Best regards,
Dr. Mahi Mokhtar

GUSTAVE ROUSSY
INTERNATIONAL

Patient Name: Fatma Abu Al Hadeed Fawzy Abu Al Hadeed	Date : 16/10/2025
Exam : CT CAP+C (CT Chest Abd. Pelvis)	Age : 43 Year(s)
Medical ID: 2758/2025	Pat. ID: 111232

MULTISCLICE C.T. EXAMINATION OF THE CHEST, ABDOMEN AND PELVIS AFTER I.V. CONTRAST INJECTION REVEALED:

Reported case of right breast cancer showing:

I- CHEST:

- Right breast ill-defined soft tissue mass lesion seen measuring about 2.7x2.9 cm associated with pathologically enlarged ipsilateral axillary lymph node largest measuring about 2.1cm in its short axis.
- No sizable pulmonary nodules, masses, consolidation, collapse or cavitary lesions.
- No sizable hilar or mediastinal masses or lymph node enlargement.
- No pleural or pericardial collection.
- No gross cardiomegaly.

II- ABDOMEN AND PELVIS:

- Average sized liver showing homogenous parenchymal attenuation. No focal lesion or dilated biliary radicles.
- Normal C.T appearance of the spleen, both kidneys, adrenal glands, pancreas, aorta and IVC.
- The urinary bladder shows no evidence of gross masses or diverticulae or stones.
- Normal appearance of the pelvic organs with no pelvic masses or collections.
- No ascites.

Opinion:

- Reported case of right breast cancer showing right breast ill-defined soft tissue mass lesion associated with pathologically enlarged ipsilateral axillary
- No CT evidence of sizable metastatic pulmonary nodules
- No CT evidence of sizable abdomen or pelvic metastases.

Dr. Nourhan Mohamed,MD

Name : Fatima Abul-Hadid Fawzi	Date : 01-11-2025 10:34
Branch : Dokki	Medical No. : 238659
Ref.Physician Prof : Gostaf Roseh Hospital	Ins.Company : تعاقد مستشفيات

$Tc^{99}m$ MDPSKELETAL SCINTIGRAPHY

History

- A 70-year-old patient, with history of breast cancer and referred for metastatic workup.

Technique

- Anterior and posterior whole-body scans were acquired 2 hours post injection of the tracer.

Findings

- No abnormal active or cold focal bony lesions that could account for metastatic bone disease.
- Degenerative changes are noted at the spine and big joints.

Impression:

- **The skeleton is free from metastases.**

Best regards,
Dr. Alaa Nagi, MD
Nuclear Medicine

NILE DIAGNOSTIC CENTER

GUSTAVE ROUSSY INTERNATIONAL
Gustave Roussy International Egypt Pathology Lab
جوستاف روسي

Patient Name	فاطمه ابو الحديد	**Case No.**	345915
Sex/Age	Female 44 Y	**Patient No.**	25517
Registered Date	04/8/2026 09:27:08 AM	**Referred By Dr.**	Surgery
Validation Date	25/8/2026 10:32:21 AM	**Branch**	Gustave Roussy
Reporting Date	25/8/2026 10:56:20 AM	**Pathology No.**	2460/26

Pathology Report

Clinical Data: Right breast cancer, Post NACT.

Nature of the specimen: Wide local excision and axillary lymph node dissection.

Gross Examination:
Wide local excision measuring 8x5x4 cm covered with skin ellipse measured 5x3 cm.
On serial sectioning revealed two wired tumor bed sites measuring 1x1 cm and 0.7x0.7 cm located 0.7 cm away from the deep margin.
Axillary lymph node dissection: Fibrofatty tissue piece measuring 8x8 cm, containing 16 lymph nodes, the largest is 1.5x1.5 cm
* **Frozen section:** Free margins
- Examined by: Dr. Walaa Ibrahim Ghoneim
* **Block designation:** 1- First TBS, 2- Second TBS, 3-7 margins, 8- skin, 9-12 lymph nodes.
* Grossing by: Dr. Aya Ezzat

Microscopic examination:

- **Histological type:** No residual viable remnants of tumor cells.
- **Ductal carcinoma in situ:** Absent.
- **Lympho-vascular emboli:** Absent.
- **Skin:** Free.
- **Lymph node involvement:** Positive for tumor metastasis in (1/16) nodes with extra-nodal extension, (0.3 cm).
- **Resection Margins:** Free margins.
- **Histologic response to therapy:** Partial pathologic response.

Diagnosis:
Right breast cancer (Post NACT), Wide local excision and Axillary lymph node dissection:
No residual viable remnants of tumor cells.
Resection margins: Free of tumor cells
Axillary lymph nodes: Positive for metastatic deposits in (1/16) with extra-nodal extension.
Stage: ypT0, ypN1
RCB= 1.205, RCB class I

Signature

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مبـــادرة رئيس الجمهورية
لدعـم صحة المرأة المصرية

08/26/2026

MDT FORM

Name:	Fatma . Fawzy .	**MRN:**	2712
Age:	44 Y	**ID:**	28205191400662
Menopausal Sta		**Phone:**	01152221597
NRS SCORE:		**Marital Status:**	MARRIED
BMI:		**Family History:**	
CO Morbidity:		**Reason Of MDT:**	POST OP

Clinical Data:

BX:

ER: 8/8 **PR:** 5/8 **HER2:** +3 **KI67:**

Sonomammography:

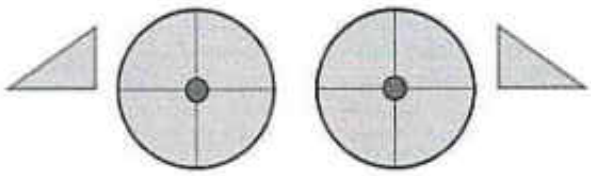
Mets Workup:

History:
PREMRT BREAST CANCER CT2 N1 MO IDC2 ER8/8 PR 5/8 HER2 3+ CTS AND BS FREE SM RT UOQ MASS 42X22 EF 70% ON 7/12 CLIPS INSERted pt rec 6 taxotere carboplatin 7 phesgo last14/6/2026 sm 21/5/2026 almoste complete radiological response level 1 indeterminate

Recommendations:
POST RT CBS +ALND , FREE MARGINS , LNS 1/16 WITH EXTRANODAL EXTENSION T0 N1

MDT Outcome & Plan
NON PCR FOR
PORTA-CTH
ADJV TDM-1 KATCYLA ONE YEAR
PORT
ZOLDEX +AI 7 YRS
ZOMETA /6 MONTHS

R L



TNM

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric
د محمد كيلاني	د محمود ابراري				

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Gustave Roussy International-Egypt- Pathology Lab
جوستاف روسي انترناشونال – مصر معمل الباثولوجي
جوستاف روسي

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Signature

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Gustave Roussy International-Egypt- Pathology Lab
جوستاف روسي للترانشونتل – مصر معمل البثولوجي
جوستاف روسي

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Dr. Walaa Ibrahim Ghoneim. MD

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